

Health Summary Report

Patient: ashiq
DOB: None
Blood Type: None

Medical History

- Hypertension
- Type 2 Diabetes
- Hyperlipidemia (elevated LDL)

Insights from Documents (AI Synthesis)

John Doe presented on March 13, 2026, with a clinical picture highly suggestive of stable angina pectoris, characterized by persistent sharp sub-sternal chest pain radiating to the left arm, lasting approximately 15 minutes, triggered by physical exertion, and relieved by rest or sublingual nitroglycerin. His medical history includes significant cardiovascular risk factors: managed hypertension and Type 2 Diabetes. Recent lab results further indicate dyslipidemia with an elevated LDL of 160 mg/dL. The current plan involves a referral for a Stress Thallium test to further evaluate myocardial ischemia and an adjustment to his hypertension management with an increased Lisinopril dosage.

Health Metrics & Vital Trends

Date	Metric	Value	Notes
2026-03-12	blood_pressure	160.0 mmHg	-
2026-03-12	blood_pressure	100.0 mmHg	-
2026-03-12	temperature	45.0 c	-
2026-03-12	weight	70.0 kg	-
2026-03-12	blood_pressure	118.0 mmHg	-

Recent Clinical Visits

Date	Diagnosis	Treatment
2026-03-08	heart attack	
2026-03-08	Possible Heart Attack, Acute Coronary Syndrome, Angina Pectoris, Myocardial Infarction	CALL 108 IMMEDIATELY, Chew and swallow aspirin (if not allergic), Sit down and rest, Loosen tight clothing, Stay calm and breathe slowly, Do not drive yourself to hospital

2026-03-04	Undifferentiated Illness — clinical assessment required	Schedule an in-person medical consultation, Keep a symptom diary to share with your doctor, Stay hydrated and rest
2026-03-04	Acute Coronary Syndrome, Angina Pectoris, Myocardial Infarction	Seek emergency care immediately (call 108), Chew aspirin 325mg if not allergic and no contraindications, Rest in a comfortable semi-reclined position, Loosen tight clothing around the chest, Monitor pulse and breathing
2026-03-04	Acute Coronary Syndrome, Angina Pectoris, Myocardial Infarction	Seek emergency care immediately (call 108), Chew aspirin 325mg if not allergic and no contraindications, Rest in a comfortable semi-reclined position, Loosen tight clothing around the chest, Monitor pulse and breathing
2026-03-04	Migraine, Tension-Type Headache, Hypertensive Headache	CALL 108 IMMEDIATELY - Time is critical, Note the time symptoms started, Do not give aspirin or other medications, Keep patient calm and lying down, Monitor breathing and consciousness
2026-03-03	Undifferentiated Illness — clinical assessment required	Schedule an in-person medical consultation, Keep a symptom diary to share with your doctor, Stay hydrated and rest
2026-03-03	Undifferentiated Illness — clinical assessment required	Schedule an in-person medical consultation, Keep a symptom diary to share with your doctor, Stay hydrated and rest
2026-03-03	Undifferentiated Illness — clinical assessment required	Schedule an in-person medical consultation, Keep a symptom diary to share with your doctor, Stay hydrated and rest
2026-03-03	Undifferentiated Illness — clinical assessment required	Schedule an in-person medical consultation, Keep a symptom diary to share with your doctor, Stay hydrated and rest

Report generated on: 2026-03-17T02:28:39.879957

This is an AI-assisted medical report synthesized from vectorized documents and clinical data. Please consult with a healthcare professional for clinical decisions.